SACTCHECK · REGIMEN-SPECIFIC SACT CONSENT

Phesgo + docetaxel

PROTOTYPE · GOVERNANCE REVIEW REQUIRED

HER2-positive metastatic / unresectable locally recurrent breast cancer · NCCP 00796 · version 4 · 21-day cycle

Patient name	DOB	Hospital no.	Date

Treatment context
Control the cancer and symptoms, with repeated review of benefit, tolerance and your priorities.

Schedule
Phesgo and docetaxel on Day 1 every 21 days. Docetaxel is planned for at least six cycles; it may stop after 6–8 cycles or toxicity. Phesgo may continue alone while beneficial and tolerated.

1. TREATMENT DISCUSSION

- ☐ Purpose, expected benefit and alternatives explained
- ☐ Duration and treatment setting discussed
- ☐ Supportive medicines and urgent contact advice discussed
- ☐ Common, serious and lasting agent-specific risks discussed below
- ☐ Correct HER2 medicine and formulation confirmed
- ☐ Heart and other required monitoring explained
- ☐ Pregnancy / contraception / breastfeeding discussed where relevant
- ☐ Companion cancer medicines reviewed using their own information

2. REGIMEN / AGENT-SPECIFIC MATERIAL RISKS

Docetaxel
TAXANE CHEMOTHERAPY

- ☐ Low blood counts, infection and sepsis
- ☐ Hair loss, mouth soreness and fatigue
- ☐ Numbness / tingling; sometimes lasting
- ☐ Fluid retention and nail changes
- ☐ Allergic infusion reaction

Pertuzumab
HER2-TARGETED ANTIBODY

- ☐ Diarrhoea and dehydration
- ☐ Injection / allergic reaction
- ☐ Heart pumping-function reduction
- ☐ Rash and fatigue

Trastuzumab
HER2-TARGETED ANTIBODY

- ☐ Heart pumping-function reduction / heart failure
- ☐ Injection or allergic reaction
- ☐ Diarrhoea and fatigue
- ☐ Rare serious lung effects

Discuss the aim, alternatives, uncertain benefit, common and serious risks, heart function, diarrhoea, injection reaction, fertility and pregnancy precautions. Effective contraception is required during Phesgo and for 7 months after the last dose; review the full combination and breastfeeding restrictions with the team. After chemotherapy stops, use the agreed Phesgo maintenance plan (NCCP 00785 when eligible). Keep cardiac and response checks; chemotherapy steroids, antiemetics and growth factors do not automatically continue.

Individual aim / benefit, alternatives, material risks and questions:

3. CONSENT CONFIRMATION

- ☐ Expected benefit, important risks and reasonable alternatives discussed
- ☐ Patient questions invited and answered; patient-specific material risks / priorities considered
- ☐ Emergency contact / toxicity advice and written information offered where appropriate
- ☐ Patient understands consent is voluntary and may be withdrawn

Patient / person giving consent _____ Date _____

Clinician obtaining consent _____ Date _____

Prototype only. This one-page form is a regimen-specific consent aid and requires formal governance, consultant and oncology-pharmacy review before clinical use. The current NCCP regimen, local/national consent policy, current medicine information, patient-specific material risks and clinical judgement remain authoritative.
Base regimen: NCCP 00796 v4.

SACTCheck v0.84.12
Developed by Dr Paul O'Brien